

kind

health exploration

We have a vision, where every individual has the best opportunity to improve their health – and to help others do the same.

Ben Fehnert

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Angel pitch · 2026

Mission, vision and values

VISION

Every individual has the best opportunity to improve their health – and to help others do the same.

MISSION

Kind brings together individuals exploring their health and the researchers building evidence to help them, creating a community where science-backed personal health exploration contributes to the evidence base.

OUR VALUES



Kind

Supportive connections that help people live well, not judgment.



Curious

Genuinely interested in what makes people tick – asking why, not just what, and following the evidence wherever it leads.



Open

Open access to research, open conversations with other explorers, and openness to ideas that challenge the mainstream.



Exploratory

Your health is personal. Try things. Track what happens. Learn.



Transparent

Transparent about how your data is used and what the research shows.

What Kind does and the principles we follow

WHAT KIND DOES

FOR INDIVIDUALS

- Choose a curated, science-backed exploration that fits your goals
- Log and track only what the exploration needs
- See whether it worked for you, and how others compare
- Opt in to contribute de-identified data to research

FOR RESEARCHERS

- Co-develop explorations as personalised (N-of-1) trials
- Reach eligible, consenting individuals through Kind
- Access de-identified data under a Data Use Agreement
- Publish findings as preprints and share them back

FOR THE EVIDENCE BASE

- Explorations follow CENT and SPENT N-of-1 standards and SPIRIT and CONSORT guidelines
- Every exploration is ethics-reviewed before launch
- Group results use pre-specified analysis plans or explicit emulative analysis
- Findings build evidence on the value of health interventions

PRINCIPLES

Logs and summaries, never advises

Kind shares what the data shows; the individual decides what to do.

Research consent kept separate

Research use is a distinct opt-in, never bundled into product consent.

Never sells individual data

No health data is sold or used for advertising.

Ethics review before launch

Protocol Review Board, then BRANY IRB, before any exploration goes live.

Intended use and indications for use

INTENDED USE · LAUNCH

Support to help an individual to understand the extent to which a specific intervention relates to a change in outcome for them.

INDICATIONS FOR USE · LAUNCH

For individual users to opt in to try to optimise or improve their health and wellbeing, by agreeing to track specific aspects of their health choices and specific (non-disease-related) aspects of their health and wellbeing. Kind shares the statistical relationship between their health choices and their health, and the user decides what they do with that information.

KIND DOES

- Matches adults (18+) to explorations using rule-based eligibility filters
- Logs and summarises self-reported and wearable data
- Shares the statistical relationship between a choice and an outcome
- Leaves every decision with the individual

KIND DOES NOT

- Diagnose, treat, cure, mitigate or prevent any disease or medical condition
- Recommend a treatment, dose or change in clinical care
- Tailor content based on self-reported conditions or medications
- Support the management of a diagnosed condition

PRODUCT STATEMENT

"Not intended for use in the diagnosis, treatment, cure, mitigation, or prevention of any disease or medical condition; not intended for use by individuals to manage a diagnosed condition or make treatment decisions."

Core team members working on Kind



**Ben Fehnert – MA,
Psychology / Design**

FOUNDER & CEO

**Founder & CEO, Kind, MA
Experimental Psychology.**

Health technology entrepreneur building services that support personal health and individual agency.



**Prof Glyn Elwyn – MD,
Clinical Decisions**

CHIEF SCIENCE OFFICER

**Professor of Health Policy,
Dartmouth Institute – US**

Pioneer of shared decision making; developed Option Grids and collaboRATE. Lead editor of the field's core textbook.



**Prof Sunita Vohra – MD,
Medicine**

SCIENCE ADVISOR

**Professor of Medicine,
University of Alberta – CA.**

Physician and senior scientist. Lead author of the landmark international consensus-based guidelines on the reporting of N-of-1 trials.



**Prof Chris Schmid –
Biostatistics**

SCIENCE ADVISOR

**Professor of Biostatistics,
Brown University – US**

Authority in statistical modelling in N-of-1 trial design and, Co-founder of the Brown Center for Evidence Synthesis in Health.



**David Haddad – MSc,
Health Technology**

HEAD OF OPERATIONS

**Public Health (UC Berkeley)
Health Policy (LSE).**

Health technologist and healthtech founder with 15 years experience building tech and AI enable healthcare in the US.



**Grady Ng – MSc,
System Design**

HEAD OF PRODUCT

**Psychology (Yale-NUS)
and HCI (UCL).**

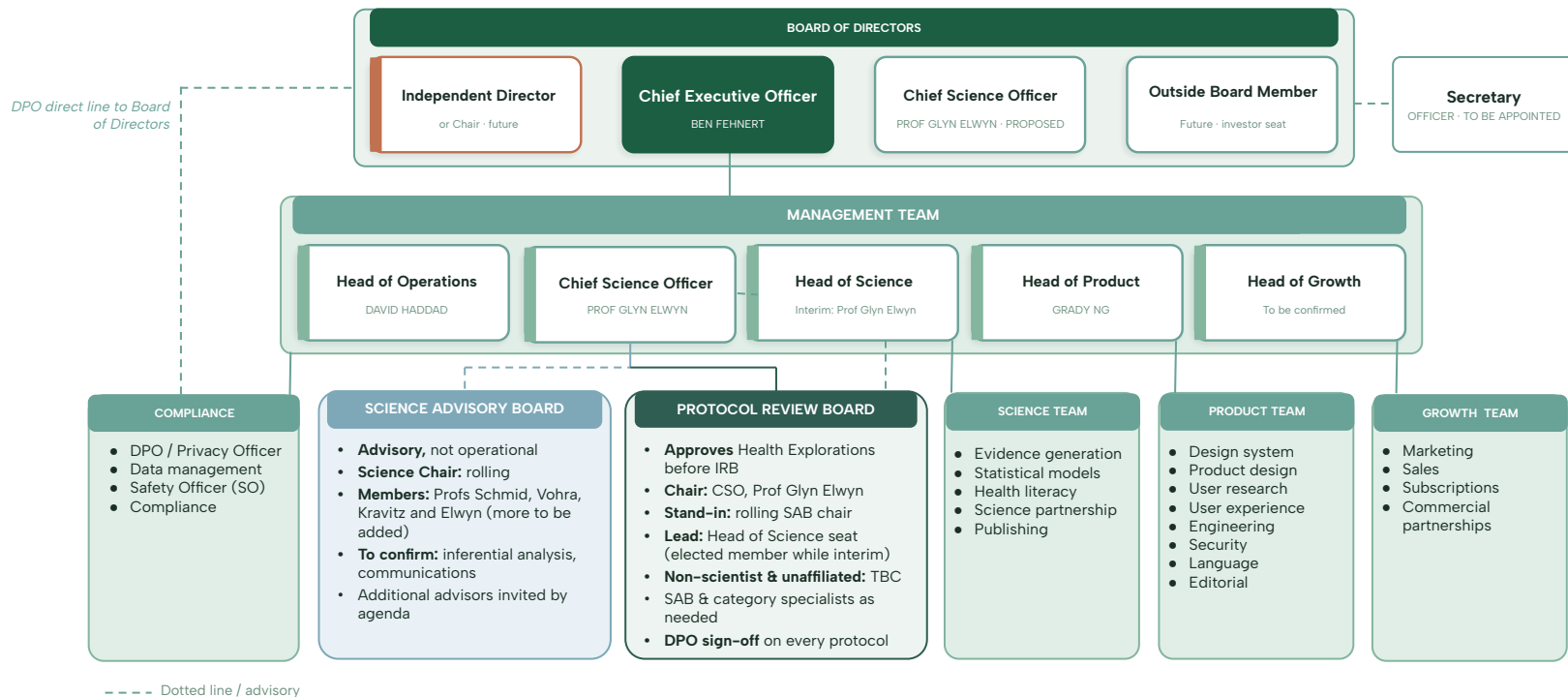
Design and Technology lead taking system design approach to Digital and AI engineering in science and healthcare.

Ben Fehnert and Prof Elwyn have collaborated extensively since 2021, most recently on Fora Health use to support mental health management in the US and in a successful Wellcome Trust AI in Health bid in the UK.

Prof Vohra and Prof Schmid are close collaborators and co-authors on the foundational Consort Extension for N-of-1 trials (CENT) Guidelines 2016 and the SPIRIT extension for N-of-1 trials (SPENT) guidelines 2020.

David and Grady both have vast experience in the iterative user centred design and build of AI enabled digital healthcare in the regulated marketplace. Both have working experience with Ben Fehnert.

05 · ORGANIZATIONAL CHART FOR LAUNCH



Board of Directors: composition and remit

COMPOSITION

AT LAUNCH

Ben Fehnert

Director · Founder & CEO

PROPOSED

Prof Glyn Elwyn

Director · Chief Science Officer

FUTURE

Outside Board Member

Investor seat, after a priced round

FUTURE

Independent Director

or Chair, after institutional funding

OFFICERS

Chief Executive Officer

Ben Fehnert

Secretary

To be appointed

Delaware C corporation: a board is required (it can be a single director) but a chair is not. Officers are appointed by the Board, one officer (the Secretary) records Board and stockholder proceedings, and one person may hold more than one office.

REMIT

- Sets strategy and approves the annual plan and budget
- Appoints and oversees the CEO and other officers, and sets their pay
- Approves financings, stock issuances and option grants
- Oversees risk, compliance and governance, including the Protocol Review Board and Compliance functions
- Receives data protection and security concerns directly from the DPO

HOW IT WORKS

1

Meets regularly: Proposed: quarterly, acting by written consent between meetings.

2

Records decisions: Directors declare conflicts; the Secretary minutes meetings and keeps resolutions.

3

Takes science and ethics input: Advice from the Science Advisory Board comes through the CSO; Protocol Review Board and IRB outcomes are reported to the Board (proposed).

Science Advisory Board: remit and process

REMIT

Advisory, not operational

- Advises the CSO and Board of Directors on science strategy, methods and integrity
- Specialist input on N-of-1 design, statistical analysis and reporting standards (CENT, SPENT)
- Provides members to the Protocol Review Board when a protocol needs their expertise
- Provides the rolling stand-in chair of the Protocol Review Board if the CSO is conflicted
- **Does not approve explorations:** the Protocol Review Board and IRB do

MEMBERSHIP

Science Chair Rolling chair	Prof Chris Schmid Bayesian analysis	Prof Sunita Vohra Methodology
Prof Rich Kravitz Science advisor	TBD Emulative trials	Prof Glyn Elwyn Decision support
Inferential analysis To be confirmed	Communications To be confirmed	Additional advisors Invited by agenda

PROCESS

- 1 Set the agenda**
The rolling chair sets the agenda with the CSO, drawing on science strategy and questions from the exploration pipeline.
- 2 Invite advisors**
Additional specialists are invited according to the agenda.
- 3 Declare interests**
Members and invited advisors declare interests at each meeting.
- 4 Advise**
Discussion and recommendations are minuted. The Board gives advice; it does not take operational decisions.
- 5 Act on advice**
The CSO takes advice into science strategy, the Head of Science into delivery, and the Protocol Review Board where relevant.

Protocol Review Board: remit and process

REMIT

- Reviews and agrees new research protocols before they are published on Kind as Health Explorations
- A documented process of review, comments, revision against approved templates and N-of-1 and trial reporting standards
- Risk assessment clinical and user-safety review of every protocol
- Document screening for the IRB; the IRB decides whether an exploration fits an approved template
- Keeps the research-ethics screen separate from product and roadmap approval (written SOP)

MEMBERSHIP

Chair: CSO, Prof Glyn Elwyn

Stand-in chair: rolling, named SAB members (if the CSO is conflicted)

Operational lead: Head of Science (elected temporary member while Prof Elwyn is interim)

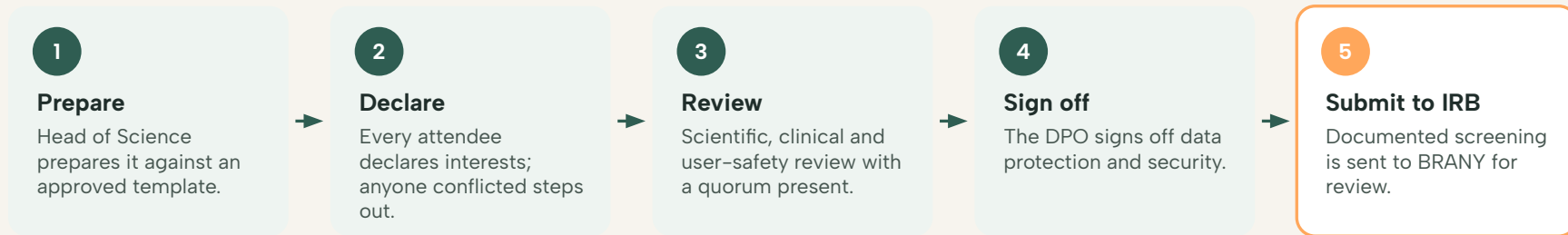
Non-scientist member: to be confirmed

Unaffiliated member: no ties to Kind, to be confirmed

As needed: SAB members and health category specialists

Quorum: chair, Head of Science, non-scientist and unaffiliated member; minimum number to confirm with BRANY.

PROCESS



Institutional Review Board (IRB): remit and process

REMIT

BRANY · central, independent, AAHRPP-accredited

- Independent ethics review and approval of every Health Exploration before launch
- Decides whether a new exploration fits an approved template
- Approves the consent model: umbrella protocol, Broad Consent for secondary analysis, specific consent per exploration
- Oversees safety: adverse-event reporting, pause triggers and audit of sub-studies

REVIEW ROUTES

Full board

Umbrella protocol and a first set of exemplar templates, reviewed first.

Expedited

A new exploration that fits an approved template, submitted as a modification.

Fuller review

A novel exploration, submitted as a modification request. Once approved it can become a new template.

Tier 1

40 of 58 catalog explorations as options for soft and full launch

US

Soft and full launch early 2027

No FDA

oversight at launch; Tier 2 and 3 are later submissions

KIND'S ACCOUNTABILITY TO THE IRB

- **Investigator of record:** Prof Glyn Elwyn (CSO)
- **Safety Officer:** adverse-event monitoring (as needed), pause triggers, reporting unanticipated problems (proposed: within 5–10 business days)
- **DPO / Privacy Officer:** data governance and consent records
- **Protocol Review Board:** documented screening with every submission

Regulatory governance

RELEASE PLAN BY EXPLORATION TIER

SOFT AND FULL LAUNCH · EARLY 2027

Tier 1 · Clinician-exempt

US only, 18+. General wellness scope, no FDA oversight. Umbrella protocol reviewed by BRANY.

LATER

Tier 2 · Clinician-informed

Excluded from the initial launch. Separate IRB submission with a consult-your-clinician gate.

SECOND LAUNCH

Tier 3 · Clinician-approved

After FDA review, likely a 510(k) as a Class II device, per treatment area.

FDA · MEDICAL DEVICE STATUS

- Launch scope is intentionally not a medical device: General Wellness and CDS guidance (Jan 2026)
- Intended use taken through FDA's step-by-step digital health review; option of a 513(g) request

Owner: Safety Officer / Compliance

HUMAN SUBJECTS ETHICS

- BRANY IRB approval before any exploration launches
- Common Rule standards adopted voluntarily; 21 CFR 50 and 56 to consider for Tier 3

Owner: CSO, as investigator of record

PRIVACY AND DATA

- Not a HIPAA covered entity or business associate; no HIPAA claims
- FTC Health Breach Notification Rule (60 days); state health data laws such as Washington MHMDA

Owner: DPO / Privacy Officer, with counsel

FTC · ADVERTISING AND SHARING

- No health data used for advertising; Kind never sells individual data
- Tier 2 and 3 flags treated as sensitive and never shared with third parties

Owner: DPO, with Head of Growth

STATE MEDICAL LICENSURE

- No diagnosis, prescription or treatment planning by any person, content or algorithm
- State-by-state review with US counsel (Wiggin and Dana), deepest for Tier 3

Owner: CEO, with counsel

ONGOING REVIEW

- Any change to intended use, scope, explorations or matching logic triggers re-review
- UK and EU (GDPR) workstream to follow the US launch

Owner: Safety Officer, CSO and DPO

Liability management by tier

ACROSS ALL TIERS

Logs and summaries only: no diagnosis or treatment advice at any tier · **Crisis escalation:** non-dismissible redirect to 911 or 988 · **Full audit trail:** consent, prompts, confirmations and screening · **Cover:** liability insurance, reviewed yearly; US counsel (Wiggin and Dana)

TIER	ENROLMENT GATE	ADVERSE EVENTS	LIABILITY CONTROLS
Tier 1 Clinician-exempt 40 in catalog · soft and full launch	- Eligibility and exclusion screening at intake - Open enrolment, no attestation	- In-app detection layer - Logged; individual directed back to their own care team	- Baseline duty of care “Not medical advice” product statement
Tier 2 Clinician-informed 9 in catalog · later	- Must confirm “I understand I should consult my clinician(s)” - Timestamped attestation	- Adds a clinician-readable summary the individual can share - Adds aggregate, pharmacovigilance-style review	- Recurring reminders through the exploration - Excludes conditions needing active supervision
Tier 3 Clinician-approved 9 in catalog · after FDA review	- Confirmed active prescription and prescriber relationship - Periodic re-attestation	- Adds live handoff to a 24/7 independent clinical triage service - Prompt to report to FDA MedWatch	- Never suggests a dose, timing or treatment change - Insurance confirmed to cover this activity

APPENDIX

A. How it works

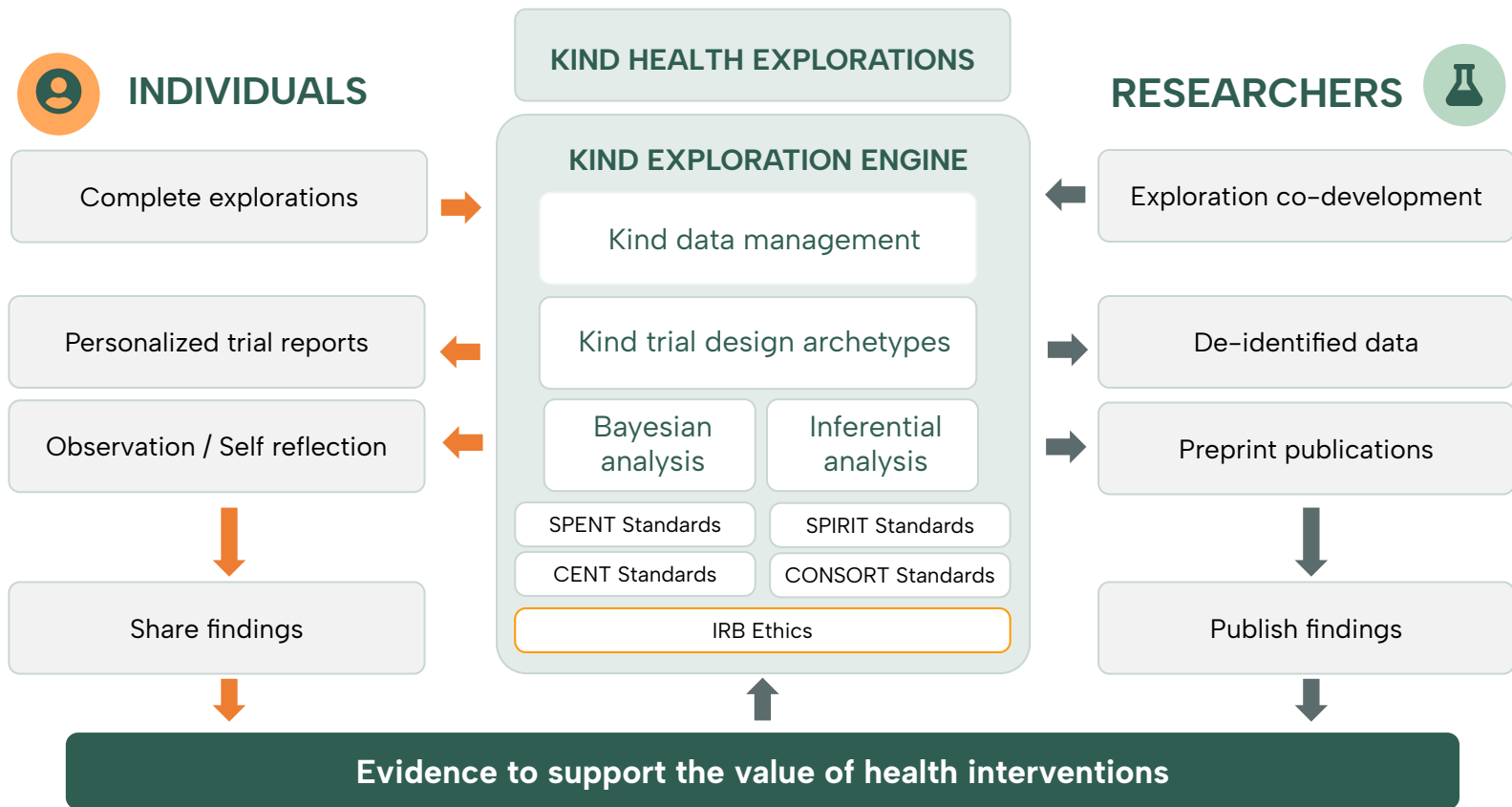
B. Health exploration examples

C. Single case protocol design archetypes

kind – health exploration

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A · HOW IT WORKS



Kind health explorations for Individuals

Category	Exploration for launch – examples	Future exploration – examples
Energy & Focus	Morning Rules for Afternoon Energy	Functional Mushrooms for Focus
Metabolic Health	Time-Restricted Eating for Energy	Breathwork for Blood Pressure
Rest & Sleep	Screen Exposure Moderation for Sleep	Cannabinoids for Sleep
Diet & Nutrition	Ultra-Processed Food Reduction for Energy	Calorie Restricted Eating for Fat %
Mental Health	Digital Boundaries: Social Media Limit	Antidepressant Response Tracking
Movement & Recovery	Cold exposure for Recovery	Mobility Exercise for Pain
Medication & Treatment Comparison	<i>Not included for launch *</i>	GLP-1 Medication & Dose Response
Women's Health	Cycle-Synced Exercise Intensity for Strength	HRT Formulation Comparison
Skin & Dermatology	Retinoid Introduction for Skin Texture	Moisturiser Type for Eczema-Prone Skin
Men's Health	Resistance Training for Energy / Mood	TRT Formulation & Dose Comparison
Allergy & Respiratory	Breathing Technique for Breathlessness	Antihistamine Type Comparison

** Kind will launch without the requirement for FDA oversight and will not include Tier 2 (clinically-informed) or Tier 3 (clinically-approved) health explorations. As a result of this there will be no 'Medication & treatment comparison' explorations at launch.*

Single case – Study protocol Design Archetypes

Single-case designs used across the Kind Explorations Catalog — matched to each exploration's reversibility, onset speed, and safety of withdrawal,

Single case archetype	Best Suited To	Exploration Catalog Examples
Alternating-Treatments (Rapid Rotation)	Fast-onset, fast-offset effects; comparing two active options day-to-day rather than in long blocks	Caffeine Timing & Offsetting Nootropic Stack: L-Theanine + Caffeine
ABAB (Reversal)	Slower but still reversible effects; replicated blocks (not just one) needed for real evidence	Breathwork for Blood Pressure Consistent Wake Time
Changing-Criterion (Ramping Target)	Cumulative, dose-like effects that build gradually rather than switch on/off	Personal Daily Step Target Zone 2 Cardio for Metabolic Fitness
Sequential Titration (No Withdrawal)	Doctor-directed medication comparisons, where stopping treatment isn't safe or ethical	GLP-1 Medication & Dose Response TRT Formulation & Dose Comparison
Split-Application (Concurrent Comparison)	Topical/localised treatments that can run on comparable body areas at the same time	Retinoid Introduction for Skin Texture Moisturiser Type for Eczema-Prone Skin
ABA (Elimination-Reintroduction)	Suspected dietary or environmental triggers, confirmed by removing then reintroducing	Elimination Trial: Dairy Dairy Reduction & Breakouts
Observational (Correlational)	Whether two things move together — no manipulation, not a causal test	Stress & Skin Flare Correlation (catalog's only current example)